Pre - Induction Checklist for Obstetric General Anaesthesia

To be read out loud during pre-oxygenation

Team

❑ **Do we need to call for any more help?**

Second Anaesthetist

1 - Bleep 063

2 - Bleep 550

ODP – Bleep 017

❑ **Do we need to contact Consultant Anaesthetist on-call?**

Patient

❑ **Has the patient received Sodium Citrate ?**

❑ **Is cardiovascular status optimal with left uterine displacement applied ?**

❑ **Is the patient in a head up / ramped position e.g using the Oxford HELP ?**

❑ **Is preoxygenation optimal?**

Use nasal oxygenation at

10L / min during intubation

Target ETO_2 > 90%

Equipment

❑ **Are necessary drugs drawn up and labelled ?**

Induction Agents

Muscle Relaxant

Opioid

Vasopressor

Atropine

Syntocinon

❑ **Is all monitoring on ?**

Capnography

Pulse oximetry

ECG & BP

❑ **Is the anaesthetist happy with the equipment ?**

Videoscope

Laryngoscopes

(short handle Mac 3 & 4)

2 ET Tubes

Bougie

Stylet

Guedel

LMA (2nd generation)

Suction to hand

Ambu bag

Difficult Airway Plan

❑ **What is the plan for a difficult Intubation ?**

A – D Blade and stylet

B – 2nd generation LMA

C – 4 handed BVM with Guedel + 2 x Nasopharyngeal Airway

D – Cricothyroidotomy

❑ **Is the relevant equipment for the above plan readily available?**

❑ **In the event of a failed intubation is the plan to wake up the patient or proceed with surgery under mask anaesthesia ?**

Refer to Table 1 – 'Obstetric Failed Intubation guideline' overleaf

❑ **Is everyone happy to proceed ?**

Proceed only if **Yes**

This checklist is not intended to be a comprehensive guide to preparation for induction

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Adapted from the 4th National Audit Project of The Royal College of Anaesthetists